

WHAT'S ACTUALLY

HAPPENING.

A straight-talk intro to hormones, perimenopause,
and what your body is actually trying to tell you.

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HORMONES

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This guide is based on personal experience and general wellness education.

It is not medical advice. Always work with your doctor for your specific situation.

CONTENTS

What's Inside

01 First, A Word

What this is and what it isn't

02 What Is Perimenopause, Actually

Not a diagnosis. A phase. Here's what it means.

03 The Four Phases of Your Cycle

What each phase feels like — and what to do with it

04 The Signs Nobody Connects

Things you've been living with that might make more sense now

05 What Actually Helps

Exercise, food, sleep, and keeping it simple

06 Adjusting Through the Month

How to modify your training and eating as things shift

07 What to Stop Chasing

The stuff that's wasting your time and stressing you out

"We're not meant to have the same body forever." — That's not a problem. That's just the truth.

WHAT'S ACTUALLY HAPPENING · A HORMONE INTRO

FIRST, A WORD

What this is and what it isn't

01

"We're not meant to have the same body forever."

01 — FIRST, A WORD

This is not a protocol. It's a conversation.

I'm not a doctor. I'm not an endocrinologist. I don't have a degree in women's hormonal health and I'm not going to pretend I do. What I have is over a decade of coaching women in their 30s, 40s, and 50s... and my own body, which has been through it.

What I kept noticing — in my clients and in myself — is that nobody had told us what was actually happening. We were exhausted, gaining weight in new places, sleeping badly, feeling anxious for no reason, losing motivation in the gym... and being told everything looked normal on paper.

This guide exists because understanding what's going on doesn't require a medical degree. It requires someone to just explain it plainly. So that's what I'm going to do.

This is not medical advice. Please work with your doctor for anything specific to your situation. This is one woman's decade of observation, lived experience, and what's actually worked.

What this guide is:

- A plain-language explanation of what perimenopause actually is and what it isn't
- A breakdown of the four phases of your cycle and what they mean for your training and energy
- The signs that most women don't connect to hormones — until they do
- What actually helps: movement, food, sleep, the basics
- How to adjust through the month instead of fighting it
- An honest list of what to stop chasing

What this guide is not:

- A hormone-balancing protocol — because balancing your hormones is not actually a thing
- A supplement guide — beyond the basics, most of it is marketing
- Cycle syncing — I don't teach it, I don't believe in overcomplicating it
- A replacement for your doctor

If you're reading this looking for a way to balance your hormones... I'd gently redirect you. Perimenopause is defined by fluctuation. Chasing balance in the middle of a phase designed around fluctuation is what's making you crazy. Let's try something else.

WHAT'S ACTUALLY HAPPENING · A HORMONE INTRO

WHAT IS PERIMENOPAUSE, ACTUALLY

Not a diagnosis. A phase.

02

"We're not meant to have the same body forever."

Nobody told us this was coming.

Perimenopause is the transition phase before menopause. It can start anywhere from your late 30s to your late 40s — sometimes earlier — and it can last anywhere from a few years to over a decade. Most women don't know it's started until they're well into it.

It's not a disease. It's not a disorder. It's a hormonal shift that's been happening to women forever. What's new is that we're finally talking about it.

Perimenopause isn't the problem. Not knowing it's happening is the problem.

What's actually changing

Estrogen and progesterone start fluctuating more erratically. Not dropping in a straight line — swinging. Some days high, some days low, sometimes both in the same week. This is why symptoms feel inconsistent and confusing. Because they are inconsistent. That's the point.

What's Shifting	What You Might Notice
Estrogen fluctuating	Hot flashes, night sweats, mood swings, irregular periods, brain fog, joint aches
Progesterone declining	Sleep disruption, anxiety, feeling wired but tired, heavier or longer periods
Cortisol sensitivity increasing	Stress hits harder, recovery takes longer, belly fat accumulates more easily
Insulin sensitivity shifting	Foods that never caused issues start to. Energy crashes. Weight changes in new places.
Muscle mass declining faster	Strength takes more effort to maintain. Body composition shifts even without diet changes.

When does it start?

Earlier than most women think. The average age is mid-40s but plenty of women start noticing changes in their late 30s. If you're 38 and things feel different than they did at 32... that's worth paying attention to. It doesn't mean something is wrong. It means something is shifting.

The fluctuation is not daily. It can be hourly. Your body is not broken. It is doing exactly what it's supposed to do — just loudly, and without warning.

WHAT'S ACTUALLY HAPPENING · A HORMONE INTRO

THE FOUR PHASES

What each phase feels like and what to do with it

03

"We're not meant to have the same body forever."

03 — THE FOUR PHASES

Your month has a shape. Learn it.

Even in perimenopause when your cycle is less predictable, understanding the general arc of your hormones gives you something to work with. You're not going to sync your life perfectly to your cycle — that's not realistic. But knowing what's coming helps you stop blaming yourself when your energy crashes or your motivation disappears.

MENSTRUAL

Days 1–5 (approx.)

Estrogen and progesterone are at their lowest. Energy is low. That's not a flaw — it's physiology.

- Rest is legitimate, not laziness — your body is doing significant work right now
- Walking, gentle movement, and light training are all fine
- This is not the week to push for new PRs or beat yourself up for not wanting to
- Iron-rich foods matter here — red meat, leafy greens, lentils
- Sleep as much as you can. Seriously.

FOLLICULAR

Days 6–13 (approx.)

Estrogen starts rising. Energy returns. This is usually when you start feeling like yourself again.

- Training feels easier — use it. This is a good week to push a little harder
- New habits stick better in this phase — if you've been meaning to start something, start it now
- Motivation is higher, mood is more stable, sleep is usually better
- Protein and carbs fuel the training well here — don't undereat in this phase

OVULATION

Peak estrogen. Peak energy. Peak strength. A short window but a real one.

- Your body is at its physical best — this is the week to lift heavy if any week is
- Coordination and reaction time are also at their peak
- Energy tends to be social and outward — channel it
- Don't let this window pass without at least one strong training session

LUTEAL

Days 17–28 (approx.)

Progesterone rises then both hormones drop toward your period. This is the phase most women struggle with most.

→ Increase your carbohydrates — your body needs more fuel in this phase and carbs help

→ Give yourself grace. Dropping to 2 sets instead of 3 is still training

→ Walking counts. Rest counts. Lighter weight counts.

→ Bloating, breast tenderness, mood changes, and food cravings are all normal here

→ Don't judge your whole relationship with fitness based on how you feel in week 4

→ Sleep often gets disrupted — prioritize it more, not less

A note on perimenopause and phase tracking:

In perimenopause your cycle becomes less predictable. Phases get longer, shorter, or skip entirely. That's normal. Use this framework as a general guide, not a rigid schedule. Pay attention to patterns over months, not days. Your body will tell you — you just have to start listening.

WHAT'S ACTUALLY HAPPENING · A HORMONE INTRO

THE SIGNS NOBODY CONNECTS

Things you've been living with that might make more sense now

04

"We're not meant to have the same body forever."

It's not in your head. It might be your hormones.

The most common thing I hear from women in their late 30s and 40s is some version of: I don't feel like myself and I don't know why. Everything looks fine on paper. My bloodwork is normal. My doctor says I'm healthy. But something is off.

Here's a list of things that might be connected that most women never put together. I'm not diagnosing anything. I'm just connecting dots that took me years to connect myself.

What You're Experiencing	What Might Be Behind It
Waking up at 3am for no reason	Progesterone decline disrupts sleep architecture — especially deep sleep and REM. It's not anxiety. It's your hormones shifting the overnight cortisol curve.
Weight gain in your belly that won't move	Cortisol sensitivity increases in perimenopause. Your body stores more around the midsection under stress — even low-grade, everyday stress.
Feeling wired and exhausted at the same time	Elevated cortisol with dropping progesterone. Your nervous system is activated but your body has no fuel to back it up.
Brain fog, forgetting words mid-sentence	Estrogen supports cognitive function and neurotransmitter production. When it fluctuates, so does your ability to access your own brain.
Anxiety that comes out of nowhere	Progesterone has a calming effect on the nervous system. As it declines, the baseline level of anxious feeling can rise without any identifiable cause.
Your workouts feel harder but you're getting worse results	Muscle recovery takes longer as estrogen declines. You need more protein, more sleep, and slightly more recovery time — not more training.
Joint aches and stiffness, especially in the morning	Estrogen has anti-inflammatory properties. Less estrogen means joints feel it more.
Periods that are suddenly heavier, longer, or irregular	Fluctuating estrogen without consistent progesterone balance creates unpredictable cycles. This is one of the earliest signs of perimenopause.
You cry at commercials or lose your temper faster than you used to	Not a personality flaw. Progesterone modulates emotional regulation. When it fluctuates, so does your emotional buffer.
Low motivation, low libido, not feeling like yourself	Testosterone — yes, women have it — also declines in this phase. It affects drive, confidence, and sex drive.

If you read this list and felt seen for the first time in years... you're not alone. Most of us figured this out way later than we should have.

WHAT'S ACTUALLY HAPPENING · A HORMONE INTRO

WHAT ACTUALLY HELPS

Exercise, food, sleep, and keeping it simple

05

"We're not meant to have the same body forever."

05 — WHAT ACTUALLY HELPS

The basics are still the basics. They just matter more now.

The internet will sell you a lot of complicated solutions to what is actually a simple problem: your body is changing and your strategy needs to change with it. The fundamentals didn't stop working. They just need to be applied with more intention.

Strength Training

This is the single most evidence-supported thing you can do for your hormonal health in perimenopause. Not yoga. Not cardio. Strength training — specifically lifting heavy enough to challenge your muscles and build or maintain lean mass.

- Muscle mass is metabolically active — it improves insulin sensitivity and helps manage cortisol
- Lifting heavy supports bone density, which declines rapidly after estrogen starts dropping
- Strength training improves sleep quality, mood, and energy — often within weeks of starting
- 3–4 days a week is enough. More is not better. Recovery is part of the work.

You are not going to get bulky. You are going to feel better, sleep better, and have a body that works for you for decades. Pick up the heavier weights.

Protein

Your protein needs go up in perimenopause, not down. Muscle protein synthesis — your body's ability to build and repair muscle — becomes less efficient with age. You have to give it more raw material to get the same result.

- Target: 0.8–1g of protein per pound of goal body weight daily
- Spread it across 3–4 meals — 30–40g per meal is the target
- Protein at breakfast matters more than most people think — it sets your hunger for the day
- Your protein and your fiber. If you only track two things, track those two.

Sleep

Sleep is not a wellness nicety in perimenopause. It is a physiological requirement that directly affects every hormone in this conversation. Cortisol resets during sleep. Growth hormone releases during sleep. Your brain clears metabolic waste during sleep. When sleep goes, everything else follows.

- Consistent wake time matters more than consistent bedtime
- Phone out of your hand at least 30 minutes before bed — not in the room if you can manage it
- Cool, dark room — estrogen fluctuations affect temperature regulation
- If night sweats are disrupting sleep, talk to your doctor — this is worth treating

Supplements Worth Knowing About

I keep this list short on purpose. The supplement industry targets perimenopausal women aggressively and most of it is noise. These are the basics with actual support:

Supplement	Why It Matters	Notes
Vitamin D3	Supports bone health, immune function, mood regulation — most women are deficient	Get your levels tested. 2,000–4,000 IU daily is common but dose to your labs.
Magnesium glycinate	Supports sleep, muscle recovery, and nervous system calm — depleted by stress	200–400mg before bed. One of the most noticeable supplements I've tried.
Omega-3 (fish oil)	Anti-inflammatory, supports brain health, joint health, and mood	1–2g EPA/DHA daily. Quality matters — look for third-party tested brands.
Creatine monohydrate	Supports muscle strength, brain function, and bone density — increasingly studied in women	3–5g daily. No loading required. Low risk, high potential upside.

Everything else — the hormone-balancing teas, the adrenal cocktails, the cortisol-fixing supplements — evaluate carefully. If it promises to balance your hormones, that's a marketing claim, not a physiological one.

WHAT'S ACTUALLY HAPPENING · A HORMONE INTRO

ADJUSTING THROUGH THE MONTH

How to modify training and eating as things shift

06

"We're not meant to have the same body forever."

06 — ADJUSTING THROUGH THE MONTH

Work with it. Not against it.

This is the practical part. You don't need a complicated protocol. You need permission to adjust... and a few simple levers to pull when things shift.

Training Adjustments

When You're Feeling...	Try This	What It's Not
Low energy, heavy, unmotivated	Drop to 2 sets instead of 3. Go lighter. Walk. Do something — anything — and call it enough.	Not failure. Not laziness. Probably luteal phase or a low-hormone day.
Strong, energized, motivated	Push. Add weight. Go for the PR. This is your follicular or ovulation window — use it.	Don't feel guilty about feeling good. Ride it.
Anxious, wired, can't settle	Lower intensity movement helps more than rest here. Walk, swim, easy bike. Hard training on a high-cortisol day digs the hole deeper.	Not the time for intervals or heavy lifting if your nervous system is already activated.
Achy, sore, joints complaining	Mobility work, lighter weights, full range of motion. Warmth before and after.	Not injury (unless it is — know the difference). Often estrogen-related inflammation.
Can't sleep, exhausted	This is not the week to add training volume. Maintain what you have. Prioritize sleep above everything.	More training will not fix poor sleep. It will make it worse.

Nutrition Adjustments

Phase / Situation	Adjustment	Why
Luteal phase (week before period)	Increase carbohydrates by one serving per day — especially around training	Your body runs hotter and burns more fuel in the luteal phase. Carbs are not the enemy here.
Low energy days	Don't cut food to compensate. Eat regular meals with protein anchored at every one.	Under-eating on low energy days makes everything worse and increases cortisol.
Night sweats / poor sleep	Avoid heavy meals within 2 hours of bed. Magnesium before sleep. Cool the room.	Digestion raises body temperature and can worsen night sweats.
Bloating and water retention	Keep sodium moderate, increase potassium (avocado, sweet potato, leafy greens), stay hydrated.	Estrogen fluctuations cause water retention. This is temporary and food-responsive.

Cravings in week 4	Don't white-knuckle it. Eat the extra carbs. Add a planned snack in the afternoon.	Progesterone drop increases cravings and hunger. Fighting it with willpower is a losing game.
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Two sets is still training. Walking is still movement. Lighter weight with good form is still building something. Stop letting perfect be the enemy of consistent.

WHAT'S ACTUALLY HAPPENING · A HORMONE INTRO

WHAT TO STOP CHASING

The stuff that's wasting your time and stressing you out

07

"We're not meant to have the same body forever."

07 — WHAT TO STOP CHASING

Stop trying to balance your hormones.

I say this with full love and zero judgment because I chased it too. For years. I followed the accounts, I bought the protocols, I tried the seed cycling and the adaptogen stacks and the specific foods at specific times. And then I realized: I was adding stress to fix a stress problem.

Perimenopause is, by definition, a phase of hormonal fluctuation. Your hormones are not going to balance. That is what this phase is. The fluctuation can be daily, hourly, weekly — driven by sleep, stress, food, your cycle, the weather, a hard conversation. There are too many variables to control and the pursuit of control is its own cortisol spike.

If an account is promising to balance your hormones, they are selling you something. Not because they're necessarily lying — but because that's not how this phase works.

The things worth letting go of:

- Cycle syncing as a rigid system — the general awareness of phases is useful, the obsessive tracking is not
- Hormone-balancing anything — teas, supplements, protocols — evaluate everything against: what is the actual claim here?
- The idea that if you just find the right protocol everything will feel like it did at 32 — it won't, and that's okay
- Punishing yourself for having a hard week in week 4 — that's physiology, not failure
- Doing more when your body is asking for less — more is not always better, especially in perimenopause
- Comparing your energy to someone else's without knowing where they are in their cycle or what phase of life they're in

What to do instead:

Learn your patterns. Not from an app that tells you what you should feel — from your own experience over months. Keep it simple. Lift. Walk. Sleep. Eat enough protein. Adjust when things shift. Come back to the basics when everything else feels like too much.

Your body is not the enemy. It is doing something hard and doing it without a manual. The more you work with it — the more you understand what it's telling you — the better this phase gets. I've seen it happen over and over. It can happen for you.

Nothing changes if nothing changes. But the change doesn't have to be complicated. It just has to be consistent.

This guide is a free download for Compass app members. If you want to go deeper on training, nutrition, or putting all of this together into a real plan, you'll find everything at femininestrengthco.com or find me on Instagram @christin_fisher. I'm glad you're here. — Christin This guide is based on personal experience and general wellness education. It is not medical advice and is not intended to diagnose, treat, or replace care from your doctor. Always work with a qualified healthcare provider for your specific situation.